DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
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Phoenix, Arizona 85016

HHS OIG CONFIDENTIAL COMPLIANCE MEMORANDUM

To: Office of Inspector General, HHS / Office of General Counsel, HHS
From: Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC, NHDP-BC, NE-BC, NPD-BC
Policy and Procedure Nurse Consultant – PX3147
Phoenix Indian Medical Center (PIMC), Indian Health Service
Date: June 10, 2025
Subject: Systemic Violation of HHS OIG Compliance Policy Standards Due to Elimination of Policy Review Function at PIMC

I. Executive Summary

This memorandum documents escalating violations of the **U.S. Department of Health and Human Services Office of Inspector General (HHS OIG) General Compliance Program Guidance (2023)** currently occurring at **Phoenix Indian Medical Center (PIMC)**, a federal healthcare entity under the **Indian Health Service (IHS)**.

Specifically:

- PIMC lacks any functioning Policy or Compliance Committee;
- My federally designated compliance oversight role (**PX3147**) was suspended by directive of the Chief Quality Officer on **May 28, 2025**;
- A compliance audit conducted on **June 9, 2025**, revealed that **ten (10) policies** activated since the governance change failed to meet legal or regulatory standards—a **100% noncompliance rate**;
- A **June 10, 2025 directive** by the Chief Quality Officer instructed staff **not to update policy expiration dates**, violating **Transmittal Notice 24-16**, the **Federal Records Act**, and HHS OIG internal control standards;
- No mechanism exists to ensure PIMC policies are aligned with federal healthcare program requirements, as mandated by OIG guidance.

II. HHS OIG Compliance Program Requirements

The **HHS OIG General Compliance Program Guidance (2023)** sets forth binding expectations for how federally funded healthcare entities must manage policy development and compliance oversight.

A. Policy Review Is Mandatory to Mitigate Risk

"When the entity creates, revises, or deletes a policy, it should consider whether the change affects the entity's compliance with government health care program requirements, encourages or incentivizes noncompliance, or impairs the entity's risk-mitigation efforts."
(*Section II.D – Policy and Procedure Management*)

Violation: PIMC is activating clinical and administrative policies without legal or regulatory review, in direct contradiction to this mandate.

B. Entities Must Maintain a System of Policy Oversight

"The Compliance Committee should ensure that a system exists to ensure that the entity's policies and procedures foster rather than undermine the entity's compliance culture."
(*Section II.D – Policy Management*)

Violation: PIMC currently has **no Policy Committee or Compliance Committee**. The **May 28, 2025 directive eliminated PX3147 review authority**, with no replacement oversight mechanism.

C. Compliance Structures Must Support Accountability

"An effective compliance program promotes adherence to the applicable requirements under Medicare, Medicaid, and other federal health care programs."
(*Section I.A – Purpose and Role of Compliance Programs*)

Violation: The policy process at PIMC currently **fosters noncompliance** by:

- Bypassing legal and regulatory review,
- Disregarding federal review cycles (CMS, TJC, HIPAA),
- Obstructing audit trail transparency by freezing expiration data updates (June 10, 2025 directive).

III. Additional Compliance Breach – June 10, 2025 Directive

On **June 10, 2025**, the Chief Quality Officer issued an instruction prohibiting staff from updating expiration dates in the hospital's policy management system (PolicyStat). This directly violates:

- **Transmittal Notice 24-16 (2024)**:

  > "All directives must be reviewed at a minimum of every 2 years... expired policies must not remain active or available for reference unless formally reapproved."

- **IHM Part 1, Chapter 1 – Internal Controls**:

  > "IHS officials must establish, maintain, and periodically review a system of internal management controls… including mechanisms for developing, reviewing, approving, and monitoring policies…"

- **Federal Records Act (44 U.S.C. §§ 3101–3107)**:
  Suppressing expiration tracking and status documentation may constitute a material misrepresentation of official records.
- **CMS Conditions of Participation (42 C.F.R. § 482.12, § 482.21)**:
  The directive removes a core function of hospital governance required under CMS certification.

This additional action compounds the risk of False Claims Act violations and signals institutional efforts to **conceal noncompliance**.

IV. Current Compliance Risks at PIMC

1. Ten (10) hospital policies activated since May 28, 2025 show **100% failure** to meet legal or regulatory standards;
2. PX3147 oversight was terminated with no alternative internal controls in place;
3. Leadership is knowingly instructing noncompliance with federal review cycles;
4. Policies affecting **billing**, **clinical procedures**, and **regulatory reporting** are being approved with no legal vetting.

This structure exposes PIMC, IHS, and HHS to:

- **False Claims Act liability (31 U.S.C. §§ 3729–3733)**;
- **Federal Records Act violations**;
- **CMS CoP enforcement** under 42 C.F.R. § 482.12 and § 482.21;
- **The Joint Commission (TJC)** accreditation jeopardy;
- **Whistleblower retaliation** under 5 U.S.C. § 2302(b)(8).

V. Requested Action

In accordance with your oversight authority under HHS policy and federal law, I respectfully request:

1. **Immediate investigation** by OIG and/or OGC into the policy approval and compliance process at PIMC;
2. **Restoration of a functioning compliance mechanism**, including PX3147 or a formal compliance review committee;
3. **Referral to the U.S. Department of Justice (DOJ)** for review of potential false statements or program fraud;
4. **Whistleblower protection** under 5 U.S.C. § 2302 and HHS policy for all personnel reporting these failures in good faith.

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